

NEXUS CLINICAL LABORATORIES

8800 Lab Drive, Suite 200, Chicago, IL 60601 | CLIA #: 14D0987654 | Tel: (312) 555-0955

LABORATORY REPORT

Patient: Samuel Brooks	DOB: January 22, 1973 (Age 40)
MRN: MRN-9911492	Gender: Female
Ordering Physician: Dr. David Lee, MD — Hematology	Accession #: NX-61163645
Specimen Type: Venous Blood / Urine	Collected: April 30, 2025 at 12:00 AM
Received: May 30, 2025 at 02:00 AM	Reported: June 21, 2025 at 05:00 AM

COMPLETE BLOOD COUNT (CBC) WITH DIFFERENTIAL

Test	Result	Units	Reference Range	Flag
WBC — White Blood Cell Count	6.5	x10 ³ /μL	4.5 – 11.0	
RBC — Red Blood Cell Count	4.23	x10 ⁶ /μL	3.80 – 5.10	
Hemoglobin	10.6	g/dL	12.0 – 16.0	L
Hematocrit	44.9	%	36.0 – 46.0	
MCV — Mean Corpuscular Volume	95.0	fL	80.0 – 100.0	
MCH	30.1	pg	27.0 – 33.0	
Platelets	220	x10 ³ /μL	150 – 400	
Neutrophils	47.7	%	50.0 – 70.0	
Lymphocytes	40.4	%	20.0 – 40.0	

COMPREHENSIVE METABOLIC PANEL (CMP)

Test	Result	Units	Reference Range	Flag
Glucose (Fasting)	112	mg/dL	70 – 99	H
BUN — Blood Urea Nitrogen	17	mg/dL	7 – 25	
Creatinine	1.05	mg/dL	0.50 – 1.10	
eGFR	92	mL/min	> 60	
Sodium	146	mEq/L	136 – 145	
Potassium	3.7	mEq/L	3.5 – 5.0	
Calcium	10.1	mg/dL	8.5 – 10.2	
AST	24	U/L	10 – 40	
ALT	32	U/L	7 – 56	
Total Bilirubin	1.1	mg/dL	0.1 – 1.2	
Albumin	3.2	g/dL	3.5 – 5.0	L

THYROID FUNCTION PANEL

Test	Result	Units	Reference Range	Flag
TSH — Thyroid Stimulating Hormone	3.05	mIU/L	0.45 – 4.50	
Free T4 (FT4)	1.13	ng/dL	0.82 – 1.77	
Free T3 (FT3)	2.6	pg/mL	2.0 – 4.4	

CLINICAL INTERPRETATION / COMMENTS

Findings are consistent with microcytic hypochromic anemia, likely secondary to iron deficiency; correlation with serum iron studies (ferritin, TIBC) is recommended. Mildly elevated fasting glucose warrants an HbA1c to evaluate for pre-diabetes. Albumin is mildly low — nutritional status assessment recommended.

Reviewed & Verified by: Dr. Michael Chen, MD | Pathology & Lab Medicine | June 21, 2025 at 05:00 AM CST | Report ID: NX-61163645